

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	Phase 1/2 Open-label Study of BMS-986466 in Combination With Adagrasib With or Without Cetuximab in Participants With KRAS G12C-mutant Advanced Solid Tumors
Short Title:	A Study of BMS-986466 With Adagrasib With or Without Cetuximab in Participants With Kirsten Rat Sarcoma Virus Glycine 12 to Cysteine (KRAS G12C)-Mutant Solid Tumors
Protocol Number:	417653501687391126
NCT Number:	NCT06024174
Sponsor Name:	Bristol Myers Squibb
Investigational Product:	cetuximab
Phase:	PHASE1, PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Part 1: Number of Participants With Dose Limiting Toxicity (DLTs): A DLT was defined as: * Death not related to disease progression * Grade (Gr) 4 (Life-threatening) neurotoxicity * Gr 3 (Severe) neurotoxicity of greater than 7 days * Gr 3 neurotoxicity does not revert to baseline within 28 days of the start date of the Grade 3 event * Seizures of grade that do not resolve within 7 days * Gr 4 cytokine release syndrome (CRS) that does not resolve to less than or equal to Gr 3 within 3 days * Gr 3 CRS that does not resolve to less than or equal to Grade 2 within 7 days * Any increase in aspartate aminotransferase (AST) or ALT $\geq 3 \times$ ULN and concurrent increase in total bilirubin $\geq 2 \times$ ULN that is unrelated to CRS and has no other probable reason to explain the combination of increases * Any other Gr 3 or 4 event deemed unexpected by the Investigator and considered a DLT upon evaluation by the safety review committee 2. Part 1: Number of Participants With Adverse Events (AEs): An Adverse Event (AE) is defined as any new untoward medical occurrence or worsening of a preexisting medical condition in a clinical investigation participant administered study treatment and that does not necessarily have a causal relationship with this treatment. 3. Part 1: Number of Participants With Serious Adverse Events (SAEs): A SAE is defined as any untoward medical occurrence that, at any dose: results in death, is life-threatening, requires inpatient hospitalization or causes prolongation of existing hospitalization, results in persistent or significant
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disability or permanent damage, is a congenital anomaly/birth defect, is an important medical event.

Secondary Obj.:

1. Part 1: Maximum Observed Plasma Concentration (Cmax)
2. Part 1: Time to Maximum Concentration (Tmax)
3. Part 1: Area Under the Serum Concentration-time Curve From Time Zero to the Time of the Last Quantifiable Concentration (AUC[0-T])

2.2 Background & Rationale

The purpose of this study is to find a safe, tolerable, and efficacious dose of BMS-986466 when given orally, in combination with adagrasib with or without cetuximab in participants with advanced KRAS G12C-mutant non-small cell lung cancer (NSCLC), pancreatic duct adenocarcinoma (PDAC), biliary tract cancer (BTC), or colorectal cancer (CRC).

3. Study Design & Methodology

Design Framework: Type: INTERVENTIONAL, Phase: PHASE1, PHASE2, Status: TERMINATED

Target N: 5

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

Key Inclusion Criteria:

Part 1:

- * Individuals with a confirmed diagnosis of advanced KRAS G12C mutant NSCLC, CRC, PDAC and BTC that has spread to other parts of the body and cannot be removed surgically, may or may not have received previous treatment with KRAS G12C inhibitors.
- * For NSCLC and CRC: Individuals must have a documented KRAS G12C mutation status from NYS or FDA approved/cleared or CE marked test or, when such result is not available, positive KRAS G12C mutation status should be confirmed by a central laboratory in blood sample collected at the time of screening.
- * For PDAC and BTC: Participants must have a documented KRAS G12C mutation from NYS or FDA-approved/cleared, or CE-marked test and blood samples will be collected only for retrospective testing.
- * Are relapsed or refractory to available standard of care treatments.

Part 2:

- * Individuals with a confirmed diagnosis of advanced KRAS G12C-mutant NSCLC (Part 2A) or CRC (Part 2B) that has spread to other parts of the body and cannot be removed surgically and have not received previous treatment with KRAS inhibitors.
- * Individuals must have a documented KRAS G12C mutation from FDA or NYS approved/ cleared or CE marked test or, when such result is not available, positive KRAS G12C mutation status should be confirmed by a central laboratory in blood sample and /or tumor samples collected at the time of screening or from archival biopsies (less than 1 year old).
- * Have failed or disease recurrence or are not able to tolerate after at least 1 previous line of therapy.

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Key Exclusion Criteria:

- * Have tumors with known BAP1 V600X, PTPN11 or KRASQ61X mutations.
- * Have or any significant heart disease or condition.
- * Receiving any medications that are substrate of CYP3A4 or inducers and/ or inhibitors

Note: Other protocol-defined inclusion/exclusion criteria apply.

4.2 Exclusion Criteria

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- * Have or any significant heart disease or condition.
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Note: Other protocol-defined inclusion/exclusion criteria apply.

11. Study Identifiers & Governance

Primary ID: 417653501687391126

Registry Number: NCT06024174

Sponsor: Bristol Myers Squibb

Study Title: A Study of BMS-986466 With Adagrasib With or Without Cetuximab in Participants With Kirsten Rat Sarcoma Virus Glycine 12 to Cysteine (KRAS G12C)-Mutant Solid Tumors

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____